

Murcha and Sanyasa

Learning Objectives

- ⇒ To know the meanings of Murcha and Sanyasa
- ⇒ To know Utpadaka Hetu of Murcha and Sanyasa
- ⇒ To know the Samprapti of Murcha and Sanyasa

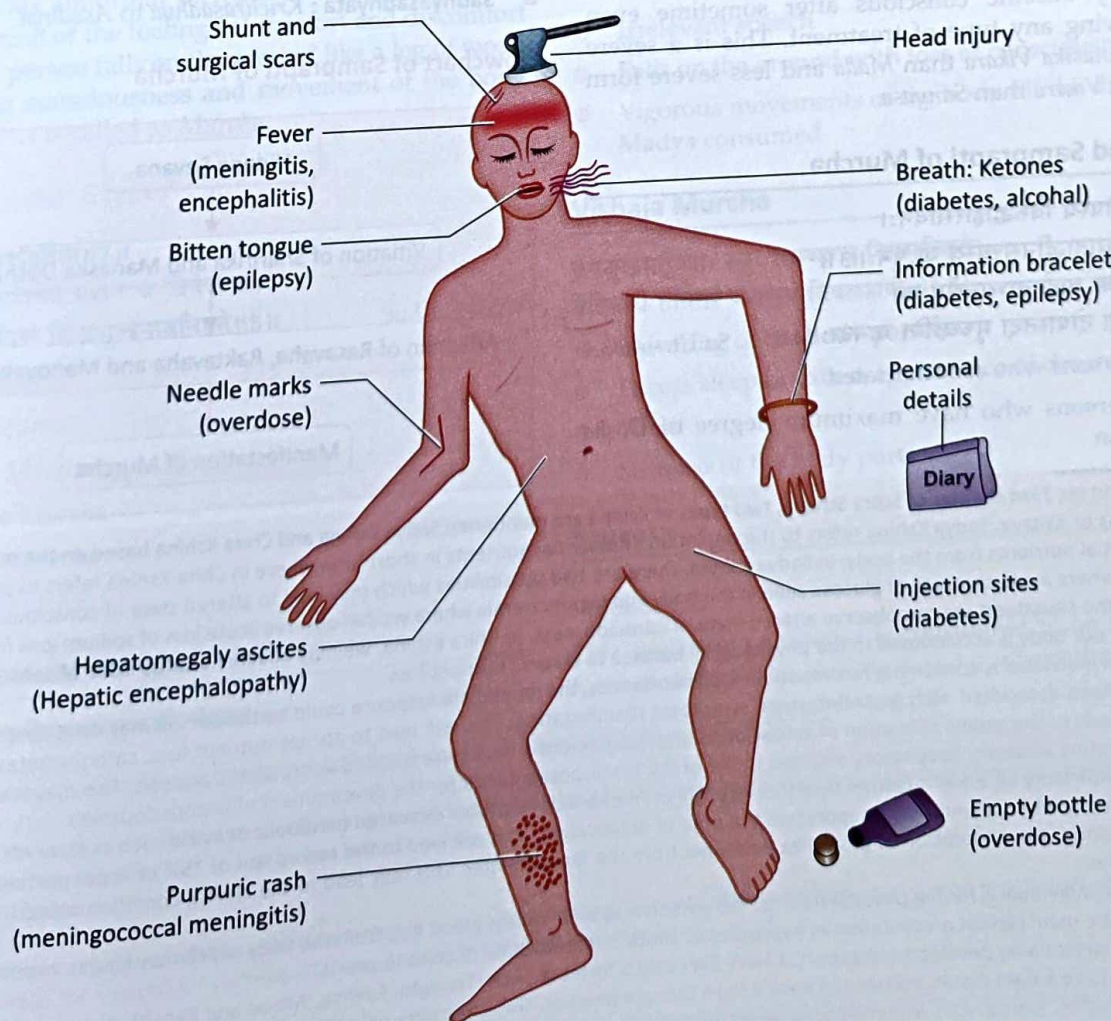


Figure 219: Nidana and Lakshana of Sanyasa

Introduction

Mada, *Murcha* and *Sanyasa* are the spectra of diseases where disturbances of functions of *Manas* can be identified, but the severity of dysfunction are seen in *Mada*, *Murcha* and *Sanyasa* in increasing severity. In all three spectra of the diseases, *Pitta Dosha* vitiation and *Tamo Dosha* vitiation can be identified. In these diseases, irritation of *Manas* and sensory impairment are mainly observed.

मदमूर्द्धायसंन्यासास्तेषां विवाद विचक्षणः।

यद्योत्तरं बलाधिक्यं हेतु लिङ्गो शान्तिषु॥१२॥

Ca.Su. – 24/12

The *Nidana* (aetiological factors), *Lakshana* (clinical features) and *Upashanti* (line of treatment) of *Mada*, *Murcha* and *Sanyasa* are same. The degrees of *Nidana* exposure, nature of clinical features and line of treatment in all three diseases are severe in *Mada*, *Murcha* and *Sanyasa* respectively. In the following pages, we are going to analyse *Murcha* and *Sanyasa* in an elaborate manner.

Murcha

Murcha is the spectrum of diseases where there will be transient loss of consciousness. In this occasion, the patient may become conscious after sometime even without giving any type of treatment. This is a severe form of *Manasika Vikara* than *Mada* and less severe form of *Manasika Vikara* than *Sanyasa*.

Nidana and Samprapti of Murcha

क्षीणस्य बहुदोषस्य विरुद्धाहारसेविनः।

वेगाघातादभिघाताद्धीनसत्त्वस्य वा पुनः॥१३॥

करणायतनेषूया बाह्येष्वभ्यन्तरेषु च।

निविशन्ते यदा दोषास्तदा मूर्च्छन्ति मानवाः॥१४॥ Su.Ut. – 46/3,4

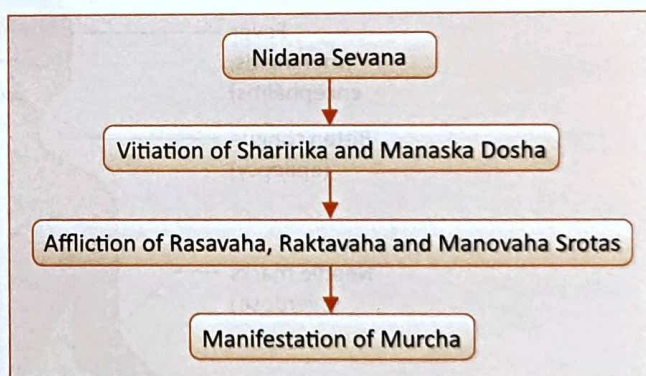
- The persons who are emaciated¹
- The persons who have maximum degree of *Dosha* vitiation

- The individuals indulged in excess consumption of incompatible substances²
- The people who frequently suppress the natural urges of the body³
- The people who are exposed to physical trauma⁴
- The persons who are having weak mind (poor mental stamina)⁵
- By exposure to above said aetiological factors, there will be excessive vitiation of morbid *Dosha*; leading to accumulation of these *Dosha* in external and internal sense organs; leading to the manifestation of *Murcha*.

Samprapti Ghataka

- *Dosha* : *Pitta*, *Rajas* and *Tamas*
- *Dushya* : *Rasa*, *Rakta*, *Manas*,
- *Srotoas* : *Rasavaha*, *Raktavaha*, *Sanjnavaha*
- *Srotodushti* : *Sanga* and *Vimargagamana*
- *Vyadhi Svabhava* : *Ashukari*
- *Adhishtana* : *Manas* and *Indriya*
- *Sadhyasadyata* : *Krichrasadhyata* to *Asadhyata*

Flowchart of Samprapti of Murcha



- Charaka in his 23rd chapter of *Sutra Sthana*, two types of *Kshina* are mentioned *Sadya Kshina* and *Chira Kshina* based on the duration of the *Kshinatva* or *Kshaya*. *Sadya Kshina* refers to the depletion of essential nutrients in short time where in *Chira Kshina* refers to prolonged loss of essential nutrients from the body. In *Sadya Kshina*, there are two possibilities which may lead to altered state of consciousness hypoglycaemia where acute shortage of glucose seen in the body and hyponatremia where we can observe acute loss of sodium ions from the body. In both the situations, we can observe altered state of consciousness. In *Chira Kshina*, we may observe altered state of consciousness rare because our body is accustomed to the physiological balance to chronic nutrient loss.
- When an individual is consuming incompatible food substances, the immediate response could be the person may develop either vomiting or diarrhoea associated with gastrointestinal symptoms manifestation. This will lead to abrupt nutrient loss, carbohydrates in particular which leads to the excess formation of ketoacidosis and lactic acidosis. This state is called as metabolic acidosis. This may lead to the state of respiratory alkalosis. Respiratory alkalosis is one of the predisposing factor for the development of unconsciousness.
S.N. : Respiratory alkalosis is caused by either hypoxia or metabolic acidosis or increased metabolic demand such as fever etc.
- For example, when an individual suppresses the urge of defaecation, that will lead to the absorption of 75% of water portion of the faecal matter along with absorption of gases like ammonia from the faecal matter. This may lead to the clinical condition called hyperammonia syndrome.
- When an individual is having physical trauma, the person may have excess blood loss from the body which may lead to hypovolemic shock. One of the main clinical presentation in hypovolemic shock is altered state of consciousness.
- Veteran personality development expert, T Harv Eker said a formula TFAR (Thought, Feeling, Action and Result). As per this formula, if you want to have a right result, you should have a right thought process. This is possible only in individuals, who are having abundance of positive thoughts. The persons who are having weak mental stamina, they are more vulnerable to negative thought processes which in turn will lead to destructive pattern of the individual. This can be even lead to altered state of consciousness.

Purvarupa of Murcha

हृत्पीडा जृम्भणं ग्लानिः संज्ञानाशो बलस्य च।
सर्वासां पूर्वरूपाणि यथाखं ता विभावयेत्॥5॥

- Pain (discomfort) in the chest region Su.Ut. – 46/5
- Excessive yawning
- Exhaustion
- Loss of consciousness (lack of knowledge of the surroundings)
- Loss of strength

Pratyatma Lakshana of Murcha

संज्ञावहासु नाडीषु पिहितास्खनिलादिभिः।
तमोऽभ्युपैति सहसा सुखदुःखव्यपोहकृत्॥6॥
सुखदुःखव्यपोहाच्च नरः पतति काष्ठवत्।
मोहो मूच्छेति तां प्राहुः

- When *Sanjnavaha Nadi* (channels carrying sensation) become obstructed by aggravated *Vata Dosha* etc leading to the enhancement of *Tamo Dosha* in the body. This abnormal increase of *Tamas* will lead to destruction of the feelings of comfort and discomfort and the person falls on the ground like a log of wood (without consciousness and movement of the body parts); this is called as *Murcha*. Su.Ut. – 46/6,7

Types of Murcha: 6 types⁶ :

षड्विधा सा प्रकीर्तिता॥7॥

वातादिभिः शोणितेन मद्येन च विषेण च।

पदस्वयेतासु पित्तं हि प्रभुत्वेनावतिष्ठते॥8॥

1. *Vataja Murcha*
2. *Pittaja Murcha*
3. *Kaphaja Murcha*
4. *Shonitaja Murcha*
5. *Madyaja Murcha*
6. *Vishaja Murcha*

Dosha dominant Murcha⁷

अपस्मारोक्लिङ्गानि तासामुक्तानि तत्त्वतः।

Su.Ut. – 46/9

4 types as per Charaka: *Vataja*; *Pittaja*; *Kaphaja* and *Sannipataja Murcha* (Ca.Su. – 24/35-41)

- 6 4 types as per Charaka: *Vataja*; *Pittaja*; *Kaphaja* and *Sannipataja Murcha* (Ca.Su. – 24/35-41)
- 7 Su.Ut. – 61/11, 12
- Vataja Apasmara* patients will have tremblings, bites his teeth, breathing discomfort; frothy vomiting; says that abnormal, black animals (or objects) are running after him and so he has lost his mind.
- Pittaja Apasmara*:
Su.Ut. – 61/12-14
- Pittaja Apasmara* patients will have thirst; warmth of the body; sweating; fainting; shakes his body parts; who is agitated; who says that yellow animals (or objects) are running after him and so one's mind becomes lost.
- Kaphaja Apasmara*:
Su.Ut. – 61/12-14
- Kaphaja Apasmara Lakshana* are coldness; nausea; excessive sleep; falls on the ground vomiting *Kapha*; who says abnormal white animal (or object) is running after him and so one's mind is lost.

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वातादिजानां तिसृणां लिङ्गमपस्मारोक्तलिङ्गातिदेशेनाह-

Dal; Su.Ut. – 46/9

As far as the types of *Vataja*, *Pittaja* and *Kaphaja Murcha* are considered, the *Lakshana* observed in these types are similar to the *Lakshana* produced in the context of *Dosha* dominant *Apasmara*.

Hence, while describing these *Dosha* dominant *Murcha*, the *Lakshana* (clinical features) mentioned in the context of *Dosha* dominant *Apasmara* should be considered.

Raktaja Murcha

स्तब्धाङ्गदृष्टिस्त्वसृजा गूढोच्छ्वासश्च मूच्छितः॥

Su.Ut. – 46/11

The patient of *Raktaja Murcha* will have

- Loss of movement of the body parts
- Loss of movement of the eyes
- Imperceptible expiration

Madyaja Murcha

मद्येन विलपंशेते नष्ट विश्रान्तमानसः।

गात्राणि विक्षिपन् भूमौ जरां यावन्न याति तत्॥12॥ Su.Ut. – 46/12

- Irrelevant speech
- Falls on the ground with loss of consciousness
- Vigorous movements of the body till the digestion of *Madya* consumed

Vishaja Murcha

वेपथुखमत्तृणाः स्युः स्तम्भश्च विषमूच्छिते।

वेदितव्यं तीव्रतरं यथास्वं विषलक्षणैः॥13॥

Su.Ut. – 46/13

- Shivering of the body parts
- Excess sleep or excess appearance of dreams
- Thirst
- Stiffness of the body parts

Sanyasa

Introduction

This spectrum of disease is considered to be the severe form of disorder of *Manovaha Srotas* where the degree of

Dosha vitiation is much more when compared to *Mada* and *Murcha*.

In this disease, the person may not regain consciousness by any of the measures, unless we give proper treatment.

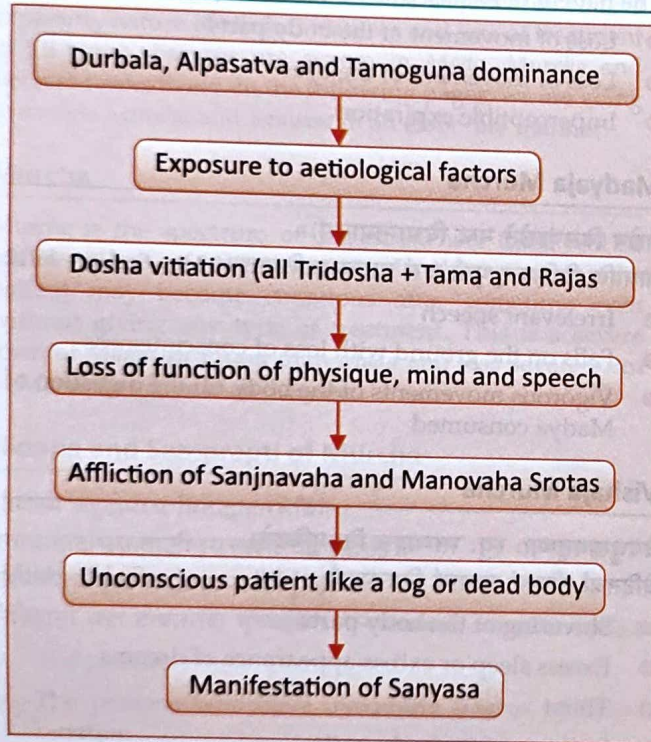
प्रभूतदोषस्तमसोऽतिरेकात् समूच्छितो नैव विबुध्यते यः॥

संन्यस्तसंज्ञो भृशदुश्चिकित्स्यो ज्ञेयस्तदा बुद्धिमता मनुष्यः।

Su.Ut. – 46/20,21

In this disease, the person will have the exacerbation of *Dosha* or dominance of *Tamo Dosha* in particular and this is characterised by non awakening state or unconscious state. This is very difficult to cure.

Samprapti of Sanyasa



Samprapti Ghataka

- ❖ **Dosha** : Tridosha + Tama and Rajas
- ❖ **Dushya** : Rakta, Mamsa, Manas
- ❖ **Srotas** : Raktavaha, Mamsavaha and Manovaha Srotas
- ❖ **Srotodushti Prakara** : Sanga and Vimargagamana
- ❖ **Vyadhi Svabhava** : Ashukari
- ❖ **Sadhyasadyata** : Krichrasadhyata to Asadhyata
- ❖ **Rogamarga** : Madhyama, Bahya

दोषेषु मदमूर्च्छायाः कृतवेगेषु देहिनाम्।

स्वयमेवोपशाम्यन्ति संन्यासो नीषधैर्विना॥ 42॥

वाग्देहमनसां चेष्टामादियातिबला मलाः।

संन्यस्यन्त्यबलं जन्तुं प्राणायतनसंश्रिताः॥ 43॥

सना संन्याससंन्यस्तः काष्ठीभूतो मृतोपमः।

प्राणैर्वियुज्यते शीघ्रं मुक्त्वा सद्यः फलाः क्रियाः॥44॥

दुर्गेऽम्भसि यथा मज्जद्वाजनं त्वरया बुधः।

गृह्णीयात्तलमप्राप्तं तथा संन्यासपीडितम्॥ 45॥

अञ्जनान्यवपीडाश्च धूमाः प्रथमनानि च।

सूचीभिस्तोदनं शस्तं दाहः पीडा नखान्तरे॥ 46॥

लुञ्चनं केशलोम्नां च दन्तैर्दशनमेव च।

आत्मगुप्तावधर्षश्च हितं तस्यावबोधने॥ 47॥ Ca.Su. – 24/42-47

In case of *Mada* and *Murcha*, the patient may regain consciousness after certain period of time.

However, in case of *Sanyasa*, until and unless we give appropriate line of treatment, the individual may not regain the consciousness.

The following features are observed in the persons who are suffering from *Sanyasa*:

- ❖ The affliction of faculties of speech, physique and mind.

The person is not moving his body parts and his mind will not respond to your stimuli and his speech is totally lost i.e. unable to listen, speak and comprehend the words spoken by other people

- ❖ The person of *Sanyasa* will look like dead person
- ❖ The person will look like log or stick like without any movement lie in the bed or in the place where he is lying

- ❖ The following measures⁸ if used judiciously and wisely, such as applying the *Anjana* (collyriums), *Avapidana Nasya*, *Dhuma Nasya* or *Pradhamana Nasya* (administering strong and potent nasal administration of medicine), pricking different parts of the body with nails, applying warm utensil to different parts of the body, pricking in between nail and nailbed, plucking hairs of the head and body; biting the patient's body with teeth, rubbing the patient's body with hairy fruits of *Atmagupta*.

Frequent administration of juice of *Matulunga* added with dry ginger and *Sauvarchala* type of salt combined with *Hingu* and wine or sour juice or vinegar should be done to arouse from unconsciousness.

- ❖ When the person regains consciousness, he should be given light diet and during this period he should be entertained with surprising narratives, memoirs, pleasing talks, enchanting songs and playing musical instruments and colourful scenery.

Moreover, he should be treated with purgation, emesis, smoking, collyrium, gargles, blood letting and rubbing over the body.

Thus, his mind should be protected well from the causes of destruction.

8 These are the measures which are considered as counter irritants which help in immediate arousal, if the person is in a state of coma.

Key summary of the chapter

- Mada, Murcha and Sanyasa are the spectra of diseases where disturbances of functions of Manas can be identified, but the severity of dysfunction are seen in Mada, Murcha and Sanyasa in increasing severity.
मदमूर्च्छासंन्यासास्तेषां विद्याद विचक्षणः। यथोत्तरं बलाधिक्यं हेतु लिङ्गो शान्तिषु॥ 1 2 ॥
- The Nidana (aetiological factors), Lakshana (clinical features) and Upashanti (line of treatment) of Mada, Murcha and Sanyasa are same. The degrees of Nidana exposure, nature of clinical features and line of treatment in all three diseases are severe in Mada, Murcha and Sanyasa respectively.
दुर्बलं चेतसः स्थानं यदा वायुः प्रपद्यते मनो विक्षोभयन् जन्तोः संज्ञां समोहयेत् तदा॥
तमेवं कश्चैवं मनो विनोभयन्नाम। संज्ञां नयत्याकुलतां विशेषश्चात्र कथ्यते॥ 1 3 ॥
- As far as Samprapti of the disease is concerned, depending upon the degree of exposure of the aetiological factor; nature of Dosha vitiation in the body; degree of affliction of Rajas and Tamas Dosha, the person may develop either Mada, Murcha or Sanyasa. When a person is indulged in contaminated food, there will be vitiation of Dosha where in vitiated Vata Dosha will afflict Hridaya (site of Chetana), deranging the functions of Manas and deranged perceptions of the affected individual. Depending upon the varied degree of abnormal Rajas and Tamas Dosha, the person may show abnormal degree of Mada.
- Sankhya Samprapti of Mada are 7 types: Vataja, Pittaja, Kaphaja, Sannipataja, Madyaja, Shonitaja and Vishaja Mada. Sankhya Samprapti of Murcha as per Sushruta are Vataja, Pittaja, Kaphaja, Shonitaja, Madyaja and Vishaja Murcha. There are no subtypes are mentioned for Sanyasa.

Ca.Su. – 24/12

Ca.Su. – 24/12,13

Self-Assessment: Questions for Review

Short essay questions

- Describe Mada
- Explain Murcha
- Describe Sanyasa
- Write the differences between Mada, Murcha and Sanyasa

Short answers

- Write a short note on Mada
- Write a short note on Murcha
- Write a short note on Sanyasa
- Enumerate the types of Mada
- Write the types of Murcha

MCQs

1. Which of the following disease is considered to be severe in severity?
 - a. Mada;
 - b. Murcha;
 - c. Sanyasa;
 - d. All of the diseases having similar in severity

Answer: (c)

2. Which of the following disease is/are considered to be mild in severity?
 - a. Mada;
 - b. Murcha;
 - c. Sanyasa;
 - d. All of the diseases having similar in severity

Answer: (a)

3. Sankhya Samprapti of Mada is
 - a. 5 types;
 - b. 7 types;
 - c. 9 types;
 - d. 3 types

Answer: (b)

4. Kashthibhuto Mritopamam is the term associated with
 - a. Mada;
 - b. Murcha;
 - c. Sanyasa;
 - d. None of the above

Answer: (c)

5. Types of Murcha as per Sushruta are
 - a. 7 types;
 - b. 6 types;
 - c. 5 types;
 - d. 4 types

Answer: (b)

6. Which of the following is used currently to determine the degree of consciousness?
 - a. EMV score;
 - b. Korotkoff sounds;
 - c. Aphasia;
 - d. All of the above

Answer: (a)

7. The best score in Glasgow Coma Scale is
 - a. 3;

- b. 10;
- c. 15;
- d. 20

Answer: (c)

8. The worst score in Glasgow Coma Scale is

- a. 3;
- b. 10;
- c. 15;
- d. 20

Answer: (a)

9. Srotas involved in Murcha is

- a. Raktavaha Srotas;
- b. Medovaha Srotas;
- c. Shukravaha Srotas;
- d. Sanjnavaha Srotas

Answer: (d)

10. Types of Sanyasa mentioned in Charaka Samhita are

- a. 3;
- b. 5;
- c. 7;
- d. No types are mentioned in Charaka Samhita

Answer: (d)

Chapter resources

- <https://neupsykey.com/the-unconscious-or-confused-patient/>
- Charaka Samhita, Sutra Sthana 24th chapter: Vidhi Shonitiya Adhyaya
- Sushruta Samhita, Uttara Tantra – 46th chapter: Murcha Pratishedha Adhyaya
- Golwalla's medicine for students by Aspi F Golwalla and Sharukh A Golwalla, edited by Milind Y Nadkar, 25th edition, Jaypee Brothers Medical Publishers (P) Ltd, New Delhi, 2017
- The Merck Manual of Diagnosis and Therapy by Robert S Porter and Justin L Kaplan, 19th edition, Merck Sharp & Dohme Corp, New Jersey, 2011
- Chamberlain's symptoms and signs in clinical medicine by Colin Ogilvie and Christopher C Evans, 12th edition, Butterworth-Heinemann Publishers, 1997
- Best & Taylor's physiological basis of medical practice by O P Tandon and Y Tripathi, 13th edition, Wolters Kluwer Health (India) Pvt. Ltd, Gurgaon, 2012
- Textbook of Medical physiology by Arthur C Guyton and John E Hall, 11th edition, Saunders Elsevier publications, Philadelphia, 2006
